

YOUR CONDITION · TENDRA

# Your spinal discs are not what you think they are.

A plain-language guide to the most misunderstood part of your spine — what discs actually do, why they wear down, and what keeps them happy.

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A short read · About 5 minutes · **For anyone who has ever heard the word "bulge" on an MRI**

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Here is what most people picture when they hear "spinal disc": a small hockey puck that can slide out of place, like a coaster knocked off a stack. The chiropractor mentioned a "bulge." An MRI report used words like "degeneration." A friend said theirs "slipped." Nobody bothered to explain what any of that actually means.

That mental picture is wrong in nearly every way that matters. And it is the reason a lot of people are quietly afraid of moving.

## 01 What a disc actually is

Between each pair of vertebrae in your spine sits a disc. There are 23 of them in total. None of them are a hockey puck.

A disc is more like a jelly donut, if the donut were made of cartilage. The outer ring is tough, layered cartilage — built like the radial layers of a tire — designed to handle compression and twisting. The center is a gel that is roughly 80% water at birth, slowly dropping over the decades. The whole structure is firmly bonded to the vertebrae above and below. It does not — and cannot — slip out like a coaster off a stack.

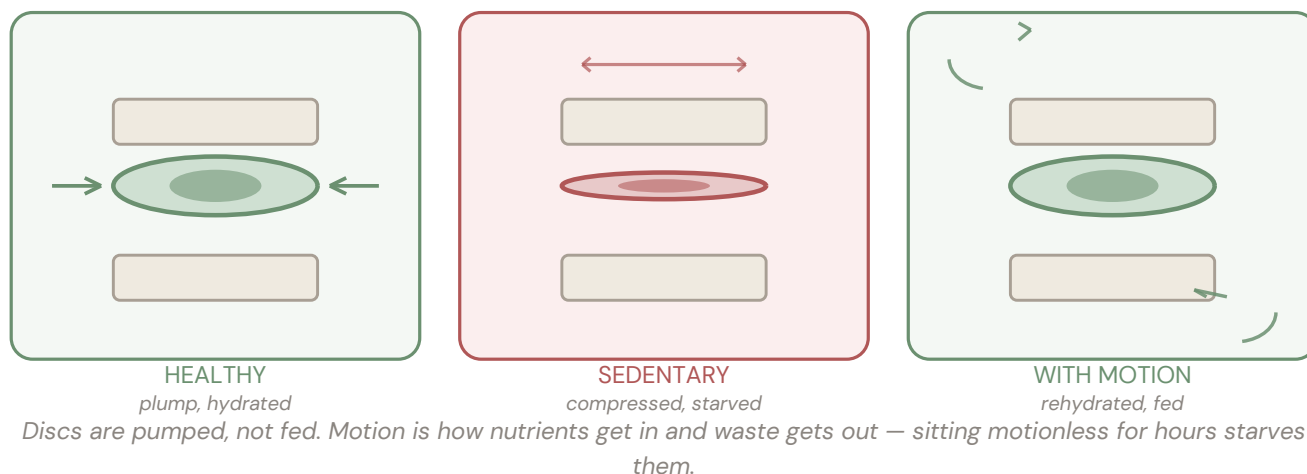
What discs do is absorb load. Every step, every cough, every time you bend to pick something up, the discs cushion the spine. They also let the spine move. Without them, the vertebrae would be one rigid column, and bending over to tie a shoe would be impossible.

## 02 How discs eat and breathe

Here is the part that almost nobody is told, and it changes how you think about everything else in this article.

Discs do not have a real blood supply of their own. That sounds like a problem — how does anything stay alive? — but it is not. The trick is that discs feed themselves through motion.

When you move, the pressure changes around the disc. Bending forward compresses the front; leaning back compresses the rear. Each compression squeezes fluid out, and each release lets fresh fluid — carrying water, oxygen, and nutrients — get sucked back in. The disc is, in effect, a sponge that has to be wrung out and re-soaked all day in order to stay alive.



### 03 What wears them down

Discs do change over time. Some of that is unavoidable; most of it is the result of habits that are surprisingly modifiable.

**Sitting for hours.** Eight hours in a chair without breaks compresses the front of the lumbar discs without ever giving them a chance to rehydrate. People who sit for a living have, on average, more disc problems than people who move for a living.

**Smoking.** One of the most consistent risk factors for early disc degeneration. The likely reason: nicotine constricts the small blood vessels at the disc edges and impairs the already-limited nutrient flow.

**Repeated bad lifting.** Lifting heavy occasionally is much less of a problem than lifting moderate weight in a bad position thousands of times. Discs handle compression well. They handle compression-plus-twist much less well.

**Dehydration and lost sleep.** Discs swell with water overnight — which is why most people are slightly taller in the morning. Chronic dehydration and broken sleep both undercut that nightly rebuild.

**Age.** Some loss of water content is normal, in the same way some loss of skin elasticity is normal. Disc "degeneration" on a 50-year-old MRI is a description, not a diagnosis. Whether it actually hurts depends on a lot more than what the picture shows.

#### IN PLAIN TERMS

Most of what damages discs is not a single bad lift. It is years of habits — long sitting, shallow sleep, dehydration, smoking — that quietly subtract from a system designed to rebuild itself nightly.

### 04 What keeps them healthy

**Move often, especially during desk work.** Stand up and walk every 30 to 45 minutes. The walking does not have to be long. The disruption of the static load is what matters.

**Stay hydrated.** Discs cannot rehydrate themselves out of nothing. Plain water through the day, not just coffee in the morning and a beer at night.

**Lift with motion in mind.** A clean, smooth movement pattern with light or moderate weight is doing your discs a favor. A wrenching, half-twisted heave with too much weight is doing the opposite. Form is a disc issue.

**Sleep on your side or back.** Stomach sleeping requires the neck to be twisted for hours and the lumbar curve to be flattened. Side-lying with a pillow between the knees is the gentlest position for most people.

**Do not smoke.** The highest-leverage one. If you do nothing else on this list, this one alone changes the trajectory.

**Discs are pumped, not fed. They have almost no blood supply — they need motion to stay alive. The eight-hour chair is not neutral. It is slow disc starvation.**

## 05 When something is genuinely wrong

Most disc-related pain is not an emergency. The vast majority of disc bulges, and even most herniations, resolve on their own over weeks and months with time, motion, and care. But a small number of presentations warrant getting someone looking sooner rather than later.

### CALL THE OFFICE OR SEEK CARE IF YOU NOTICE

- **Numbness in the groin or saddle area**, or new trouble controlling the bladder or bowels.
- **Progressive weakness in a leg** — actually losing strength, not just a passing tingle.
- **Pain shooting down one leg** that is getting worse over weeks rather than better.
- **A sudden, severe injury** that produced new severe pain.
- **Pain that wakes you up at night** and cannot be relieved by any position.

None of these mean a disc is "destroyed." For everything else: keep moving, stay hydrated, sleep well, and trust the body to do what it has done for the rest of vertebrate history.

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**About this guide.** This is general patient education and is not medical advice for any specific person. Spinal discs and the conditions affecting them vary considerably between individuals — any concerns about a specific symptom or imaging finding should go directly to the treating chiropractor or a relevant medical provider. If symptoms feel like a medical emergency, call 911 or go to the nearest emergency department.